



INPATIENT RECORD

Hospital No: 67053504	Visit No:
Name:	Age/Sex:
Doctor Name:	Specialty:
Date:	

SPO2: 99 %	BP Systolic: 140 mmHg	BP Diastolic: 80 mmHg
Pulse Rate: 78 bt/min	Respiration: 20 br/min	Temperature: 97.6 °F
Oxygen Delivery Device: RA	Saturation(Oxygen): 97 %	Mean Arterial Pressure-MAP: 100 mmHg
BP Systolic: 130 mmHg		

Forms

Fall Risk Assessment Tool

Total Fall Risk Score(sum of all points per category) : 9
Age : 70-79 Years - 2

Medications:

ON 2 OR MORE HIGH FALL RISK DRUGS - 5

Mobility : Requires assistance or supervision for mobility,transfer or ambulation - 2

Pain Assessments

Score: 1

Type of Assessment: Shift Assessment

Pain Assessment Scale: Numeric Pain Scale (0-10 Scale)

Numeric Pain Scale: 1

Additional Pain Assessment Details

Type: Not Applicable

Duration: Not Applicable

Continuous: Not Applicable

Time: Not Applicable

Associated Symptoms: None

Questionnaires

Status : Entered

Date : 25/03/2026



Hospital No: 67053504
Name:

Visit No:
Doctor Name:

Sex : Male

A - Each Risk Factor Represents 1 Point : Other risk factor

Total Tickmarks : 1

Total Score A : 1

C - Each Risk Factor Represents 3 Point : Age over 75 years

Total Tickmarks : 1

Total Score C : 3

Total Score (A+B+C+D+E) = : 4

Intravascular Access Lines

Status : Entered

IntraVascular Access Type : Peripheral IV Line

Peripheral IV Activity : Site assessed

CVC Site Assesst

Lumen color : PINK

Central IV TPA Intervention

Catheter position corrected / withdrawn

Arterial Line Status

Peripheral IV Assessment

Adult Pressure Ulcer Braden Scale

Status : Entered

SENSORY PERCEPTION : 4

4 - No Impairment : Y

MOISTURE : 4

4 - Rarely Moist : Y

ACTIVITY : 1

1 - Bed Bound : Y

MOBILITY : 3

3 - Slightly Limited : Y

NUTRITION : 3

3 - Adequate : Y

FRICTION AND SHEAR : 2

2 - Potential Problem : Y

TOTAL SCORE : 17

Status : Entered

ADULT & PAEDIATRIC - ASPIRATION RISK ASSEMENT TOOL

Status : Entered

Score : 0

CATEGORIES

No Risk - 0 : Y

NURSING INITIAL ASSESSMENT - NEW

Status : Entered



Hospital No: 67053504

Visit No:

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Doctor Name:

Score : 0
Patient Name : Mr Kishore CHOUDHURY
MRN : MH018148483
Age : 75yrs
Gender : male
Patient arrival Date : 25/03/2026
Patient Arrival Time : 11:55
Accompanied by Relative : Yes
Name of Relative : MR KUNAL CHOUDHURY
Relationship with Patient : SON
Contact Number : 9831026272
Primary Language Spoken : bengali
Room Orientation Given : Yes
Patient Identification Band Fixed : Yes
If Yes Type Of Band : white
Height(Cm) : 168
Weight(Kgs) : 64
Reason Not Able To Check : all checked
Chief Complaints : shortness of breath/ loss of consciousness
Past History : HTN, T2DM, COPD/ POST CABG
Vital Assessment
Temp(F) : 98.2
Pulse/min : 64
Blood Pressure(mmHg) : 120/80
Respiration/min : 18
Spo2(%) : 97
GRBS (mg/dl) : 114
Skin Integrity : intact
Other Parameters
GCS : 15/15
Pain Score : 1/10
Valuable handover checklist completed : Yes
Belongings : Yes
Dentures : No
Hearing Aid : No
Eye Glasses/Contact Lens : No
Vulnerable : Yes
Any Special Care Given : keep close monitoring
Restraints : No
If Yes, Type Of Restraint : na
Any Known Allergies : not known
Current Medications : GLYCOMET SR 850MG--OD
AB FLO 100MG--BD
NEBICARD 5MG--OD
ROSUVAS 10MG--ODHS
DEPLATT 75MG--OD
PRUSENT 2MG ---OD



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ACTONIL 5MG --OD

TELMA 80MG--OD

NEBS WITH DUDECORT // LEVOLIN

Informed the Doctor : Yes

Name of the Doctor : DR. TRINA MANNA

Investigations Ordered : blood investigation send from emergency

Diet : SR DIABETIC

Collected Old records : No

Is the patient at risk for any of the following? If Yes, complete the risk assessment form.

Fall : YES

Pressure Score : NO

DVT : NO

Aspiration : NO

Pain Assessment Score : Not Applicable

Ability to perform activities of daily living

Activity

Bathing : Assisted

Dressing : Assisted

Eating : Assisted

Walking : Assisted

Toilet Use : Assisted

Nursing needs(complete Nursing care plan based on the identified needs)

Actual needs : loss of consciousness, shortness of breath

Potential needs : keep close monitoring

Assessment Done By : SILPI SAPUI

Rating : 0

BMI : 22.68

General Physical Examination:

Pallor : NO

Icterus : NO

Edema : NO

Clubbing : NO

Cyanosis : NO

Lymphadenopathy : NO

Any Other Findings : itching spot buttocks.

IP Initial Assessment

Status : Entered

Score : 0

Patient Arrival Date : 25/03/2026

Patient Arrival Time : 12:30

Chief Complaints : PATIENT C/O TWO EPISODES OF TRANSIENT LOSS OF CONSCIOUSNESS . ONE EPISODE YESTERDAY AND ONE EPISODE TODAY (BOTH EPISODES AFTER BREAKFAST)

PATIENT NOW C/O EXTREME WEAKNESS AND UNABLE TO EAT

HE WAS ADMITTED TO A DIFFERENT NURSING HOME WITH AECOPD AND LRTI (HE WAS DISCHARGED ON 23/03/26)

HTN

T2DM



Hospital No: 67053504

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Doctor Name:

POST CABG
COPD

History of Presenting illness : PATIENT C/O TWO EPISODES OF TRANSIENT LOSS OF CONSCIOUSNESS .
ONE EPISODE YESTERDAY AND ONE EPISODE TODAY (BOTH EPISODES AFTER BREAKFAST)
PATIENT NOW C/O EXTREME WEAKNESS AND UNABLE TO EAT
HE WAS ADMITTED TO A DIFFERENT NURSING HOME WITH AECOPD AND LRTI (HE WAS
DISCHARGED ON 23/03/26)

HTN
T2DM
POST CABG
COPD

Allergy : NOT KNOWN
Past Medical History : HTN
T2DM
POST CABG
COPD
HFpEF

Current medications : TAB GLYCOMET SR 850-ONE TAB ONCE DAILY AFTER LUNCH X CONT
TAB MONTEK FX -ONE TAB ONCE DAILY AT BEDTIME X CONT
TAB ABFLO 100-ONE TAB TWICE DAILY AT 9AM AND 9 PM X CONT
NEBULIZER DUOLIN -THRICE DAILY X CONT
NEBULISER BUDECORTTWICE DAILY X CONT
TAB DEPLATT 75-ONE TAB ONCE DAILY AT BEDTIME X CONT
TAB ROSUVAS 10-ONE TAB ONCE DAILY AT BEDTIME X CONT
CAP DV 60K -ONE CAP ONCE WEEKLY X 2 WEEKS(ON SATURDAYS)
SYRUP BEVON 10 ML-THRICE DAILY X 6 WEEKS
TAB PRUSCENT 2MG-ONE TAB ONCE DAILY AT BEDTIME X CONT
TAB ACTONIL 5 MG-ONE TAB ONCE DAILY AT BEDTIME X CONT
TAB PANTOP 40-ONE TAB ONCE DAILY BEFORE BREAKFAST X CONT
TAB TELMA 80-ONE TAB ONCE DAILY AFTER BREAKFAST X CONT
SYRUP DUPHALAC 15ML -ONCE DAILY AT BEDTIME X CONT
TAB LOXOF 750-ONE TAB ONCE DAILY X 5 DAYS

Pulse /min : 64
Resp Rate/min : 18
Temp F : 98.2
BP mm Hg : 120/80
Room Air : Y
GRBS mg/dL : 114
JVP : NOT RAISED
Pallor : NO
Icterus : NO
Edema : NO
Clubbing : NO
Cyanosis : NO



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Lymphadenopathy : NO
CVS : S1,S2 AUDIBLE
RS : B/L VBS+
PA : SOFT,NON TENDER,IPS+
CNS : NO FOCAL NEURODEFICIT
Others : GENERALISED WEAKNESS
Provisional Diagnosis : SYNCOPE UNDER EVALUATION
AECOPD
GENERALISED WEAKNESS
HTN
T2DM
Care Plan : MEDICAL MANAGEMENT
72 HOURS WEB CARDIO

TO DECIDE IF PPM IMPLANTATION NEEDED
Spo2 : 97
Diet Plan : SALT AND FAT RESTRICTED DIABETIC DIET

Progress Note :

H/O SYNCOPE . POST CABG STATUS .
ADV : ELECTROLYTES .
NA K MG , TSH T4 , D DIMER
48 HRS WEB CARDIO
CAROTID DOPPLER

REFER : DR B PAUL NEURO .

TAB VERTIM OD 24 ONCE A DAY
TAB BREVIPIL 50 BD

ADV : CTCAG AND GRAFT ANGIO

Progress Note :

48 TO 72 HRS WEB CARDIO .

Residents Notes :

PATIENT C/O TWO EPISODES OF TRANSIENT LOSS OF CONSCIOUSNESS . ONE EPISODE YESTERDAY AND ONE EPISODE TODAY (BOTH EPISODES AFTER BREAKFAST)
PATIENT NOW C/O EXTREME WEAKNESS AND UNABLE TO EAT
HE WAS ADMITTED TO A DIFFERENT NURSING HOME WITH AECOPD AND LRTI (HE WAS DISCHARGED ON 23/03/26)
HTN
T2DM
POST CABG



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COPD

CAROTID SENSITIVITY TEST ,DONE IN ED =++(HR WENT DOWN TO 32/MIN)

CURRENT VITALS-

T-98.2

HR-64/MIN

RR-18/MIN

SPO2-95% RA

BP-120/80 MM

CBG-114 MG/DL

C/D/W-DR S K DUTTA SIR-

WEB CARDIO 72 HOURS

2D ECHOCARDIOGRAPHY

SEROLOGY TEST

HOLD TAB NEVICARD FOR NOW

PLEASE DO NOT LET OUT OF BED ALONE/WITHOUT ASSISTANCE

CHECK HR REGULARLY AND INFORM STAT IF HR<= 40

CHECK CBG TDS AND INFORM

CONTINUE OTHER MEDICATIONS AS ADVISED

FLUID RESTRICTION OF 1.5L/DAY

GIVE SALT AND FAT RESTRICTED DIABETIC DIET

CHASE REPORTS

INFORM SOS

Forms

Nurses Endorsment Checklist

Status : Entered

Shift : Morning to Evening

Situation : Shift Handover

Comment / Chief Complaints / Diagnosis : HTN

T2DM

POST CABG

COPD

Background

Allergies : No

Diet : Yes

Previous Medications : Yes

Old reports added to TRAK : No

Consents : Yes

Cross Consultation : No

Procedures performed before transfer : No

Vulnerable : Yes

Past Medical / Surgical History : TN

T2DM

POST CABG



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COPD
HFpEF
Level of Consciousness : Oriented
GCS : alert
Vitals -TPR & BP : is stable : Yes
Pain Score : 2/10
On MET/PET List : No
Risk For FALL : Yes
Risk For Pressure Ulcer : No
Risk for Aspiration : No
Existence of Pressure Ulcer : No
O2 Administration : No
SPO2(Latest reading in the shift) : 99
DIET : Oral
Diet-Specify the type : Normal
On tube feeding : No
IV Fluids : No
Blood/Blood products : No
Total Parenteral Nutrition : No
Pre Antibiotic Culture : No
Invasive Lines : Yes
Self Void : Yes
Foley's : No
Drain : No
Bowel Opened : Yes
Is the patient posted for any Surgery / Procedure : No
b)If Yes, is the financial clearance obtained (As applicable) : a)If Yes, Refer the pre-operative checklist
Dressing Present : No
High Risk medications : No
Is Patient on Physiotherapy : No
Ambulation : Ambulant
Investigations Sent : No
Reports Collected : No
Care Plan : Yes
Special Needs : No
Patient & Family Education : Yes
Transfer : No
Medications: Yes
Planned Procedure : No
Approved follow-up : No
Transfusion : No
Patient belongings with health care providers : Yes
Pending Reports : No
Critical Value : No
Pharmacy / Consumables : Charged
Discharge Plan : N/A
Comments : web cardia 72 hour



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Doctor Name:

Hand over by : SILPI SAPUI

Handed over to : GOURANGA MAITY

Team Lead Hand over By : GYOTSNA RANI MONDAL

Team Lead Hand over To : SAYANTIKA SAMANTA

Status : Entered

Add Nursing Care Plan

Status : Entered

Nursing Care Plan discussed with Patient : Yes

Nursing Care Plan discussed with Family : No

Patient Measurable Goal for the day : to reduce anxietyNursing Diagnosis : Anxiety

Education and comments: : maintain hygiene

Nursing Interventions : given psychological support

given medication as per DR. advice

Evaluation : Continue

Initial Nutritional Assessment

Status : Entered

ANTHROPOMETRIC MEASUREMENTS

Height (cms) : 168

Weight (kgs) : 64

IBW (kgs) : 64.63

BMI : 22.68

Recent weight loss/gain : None

GI SYMPTOMS (Recent)

None : Y

Food allergies/Intolerance : No

Subjective Global Assessment rating

Well Nourished : Y

Nutritional intervention proposed/Diet advised : Soft diet, diet progression as per medical condition

Others : SOFT DIABETIC DIET.

LOW SALT+FAT.

PT CH SOFT

Select Type : Assessment

Evaluation Criteria

Adult Others:DM.HTN.COPD.POST CABG.? SYNCOPPE.

Well Nourished

SGA RATING : A

Forms

Fall Risk Assessment Tool

Total Fall Risk Score(sum of all points per category) : 8

Fall History : One fall within 6 months before admission- 5

Mobility : Requires assistance or supervision for mobility,transfer or ambulation - 2



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Cognition : Altered awareness of immediate physical environment- 1
Type of Assessment : Shift Assessment
Pain Assessments
Score: 2
Type of Assessment: Shift Assessment
Pain Assessment Scale: Numeric Pain Scale (0-10 Scale)
Numeric Pain Scale: 2

Additional Pain Assessment Details

Since: 25/03/2026
Type: Not Applicable
Duration: Not Applicable
Continuous: Not Applicable
Time: Not Applicable

Associated Symptoms: None

ADULT & PAEDIATRIC - ASPIRATION RISK ASSEMENT TOOL

Status : Entered
Score : 0
CATEGORIES

TOTAL SCORE : 2
Patient Acuity Tool
Stable patient 1 to 11
Moderate Risk Patient 12 to 22
Complex Patient 22 to 33
High Risk Patient >34

Total Score :22
ADULT & PAEDIATRIC - ASPIRATION RISK ASSEMENT TOOL
Status : Entered
Score : 0
CATEGORIES

TOTAL SCORE : 2
Adult Pressure Ulcer Braden Scale
Status : Entered
SENSORY PERCEPTION : 4
4 - No Impairment : Y



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MOISTURE : 4
4 - Rarely Moist : Y
ACTIVITY : 4
4 - Walks Frequently : Y
MOBILITY : 4
4 - No Limitations : Y
NUTRITION : 4
4 - Excellent : Y
FRICTION AND SHEAR : 3
3 - Apparent Problem : Y
TOTAL SCORE : 23
Status : Entered
Questionnaires
Status : Entered
Date : 25/03/2026
Sex : Male
C - Each Risk Factor Represents 3 Point : Age over 75 years
Total Tickmarks : 1
Total Score C : 3
Total Tickmarks : 4
Total Score (A+B+C+D+E) = : 3
Risk Assessment : : Low
Intravascular Access Lines
Status : Entered
IntraVascular Access Type : Peripheral IV Line
Peripheral IV Activity : Site assessed
CVC Site Assesst

Lumen color : Pink
Central IV TPA Intervention

Catheter position corrected / withdrawn

Arterial Line Status

Peripheral IV Assessment
Need Assessment :
Yes
Care : Cap(s) changed, Skin cleansed with chlorhexidine, Secured with steri-
strips
Site Details : No
complications
Dressing Type / Appearance :
Intact
Peripheral IV Patency : No complications

Peripheral IV Drainage Present : No



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Name:

Doctor Name:

Forms

Add Nursing Care Plan

Status : Entered

Nursing Care Plan discussed with Patient : Yes

Nursing Care Plan discussed with Family : No

Patient Measurable Goal for the day : Will be improved daily activity

to prevent hospital acquired infection Nursing Diagnosis : Risk for fall

Education and comments: : maintain oral Hygiene.

PLAN FOR CT ANGIO

Nursing Interventions : to keep side rails up

to provide comfort and maintain privacy

to reduce risk for fall

Evaluation : Continue

Residents Notes :

RECURRENT SYNCOPE

RBBB

AECOPD

WEB CARDIO ONGOING

ECHO - PRESERVED EF

1 EPISODE OF SOB

CHEST RHONCHI +

INJ HCORT 200 MG IV STAT

START NIV

PULMO OPINION

ABG STAT

MAY REQUIRE ICCU TRASFER

Investigation Orders**Laboratory :** - Magnesium - Serum

- Thyroid Profile (T3 T4 Tsh)

- SEROLOGY PROFILE (HIV I & II, HBsAg, HCV)

Procedure : - ABG Analysis

- Web Cardio (3 Days) (WEB CARDIO 72 HOURS)

- 2 D Echocardiography

MEDICATION ORDERS

Drug / Generic Item	Dosage Qty	Frequency	Instructions/Notes	Duration
GLYCOMET-SR 850 MG SUSTAINED RELEASE TABLET (METFORMIN (80006846))	1 TAB	ONCE	AFTER LUNCH	5 DAY(S)
MONTEK-FX (120mg, 10mg) tablet [10] (FEXOFENADINE~MONTELUKAST)	1 TAB	NIGHT DOSE(NIGHT DOSE (HS))		5 DAY(S)
AB PHYLLINE 100 MG CAPSULE 15'S (ACEBROPHYLLINE (87730))	1 CAPSULE	12 HOURLY (Q12H)		5 DAY(S)



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DEPLATT 75 MG TABLET (CLOPIDOGREL (80003102))	1 TAB	NIGHT DOSE(NIGHT DOSE (HS))		5 DAY(S)
ROZAVEL 10 MG TABLET 15'S (ROSUVASTATIN (80013586))	1 TAB	NIGHT DOSE(NIGHT DOSE (HS))		5 DAY(S)
LUMIA 60000IU CAPSULES (VITAMIN D (80011282))	1 CAPSULE	ONCE		1 WEEK(S)
BEVON ORAL SUSPENSION 200 ML (RETINOL~VITAMIN D~THIAMINE~RIBOFLAVIN~PYRIDOXINE~NICOTINAMIDE~VITAMIN B12~DEXPANTHENOL~ZINC SULFATE~IODINE~MOLYBDENUM~MANGANESE~SELENIUM~LYSINE~BETACAROTENE~BIOTIN~CHROMIUM~INOSITOL)	10 ML	EVERY 8 HRS(8 HOURLY (Q8H))		5 DAY(S)
PRUVICT (2mg) tablet [10] (PRUCALOPRIDE (80008681))	1 TAB	NIGHT DOSE(NIGHT DOSE (HS))		5 DAY(S)
ALTONIL 5 MG TABLET (MELATONIN (7453))	1 TAB	NIGHT DOSE(NIGHT DOSE (HS))		5 DAY(S)
PRASOPHEG 40 MG TABLET 10'S (PANTOPRAZOLE (80007606))	1 TAB	ONCE		10 DAY(S)
TELMA 80 MG TABLET (TELMISARTAN (80009973))	1 TAB	ONCE		10 DAY(S)
DUPHALAC BULK (10g) oral solution [160ml] (LACTULOSE)	15 ML	NIGHT DOSE(NIGHT DOSE (HS))		10 DAY(S)
LOXOF 750 MG TABLET (LEVOFLOXACIN (80006196))	1 TAB	ONCE		5 DAY(S)
FORACORT 1 MG RESPULES INHALATION SOLUTION 2 ML (FORMOTEROL~BUDESONIDE)	1 PC	12 HOURLY (Q12H)		7 DAY(S)
NEBZMART G 25 MCG/2 ML SMARTULES (GLYCOPYRRONIUM)	1 INHALATION SOLUTION	12 HOURLY (Q12H)		7 DAY(S)
CORT-S 100 MG/VIAL POWDER FOR INJECTION (HYDROCORTISONE (80005336))	50 MG	STAT		
CAPRIN (25000IU) solution for injections [5ml] (HEPARIN (80005278))	1 VL	ONCE		1 DAY(S)
MAGNEON (50% w/v) solution for injections [2ml] (MAGNESIUM SULFATE (80006731))	1 AMP	ONCE		1 DAY(S)